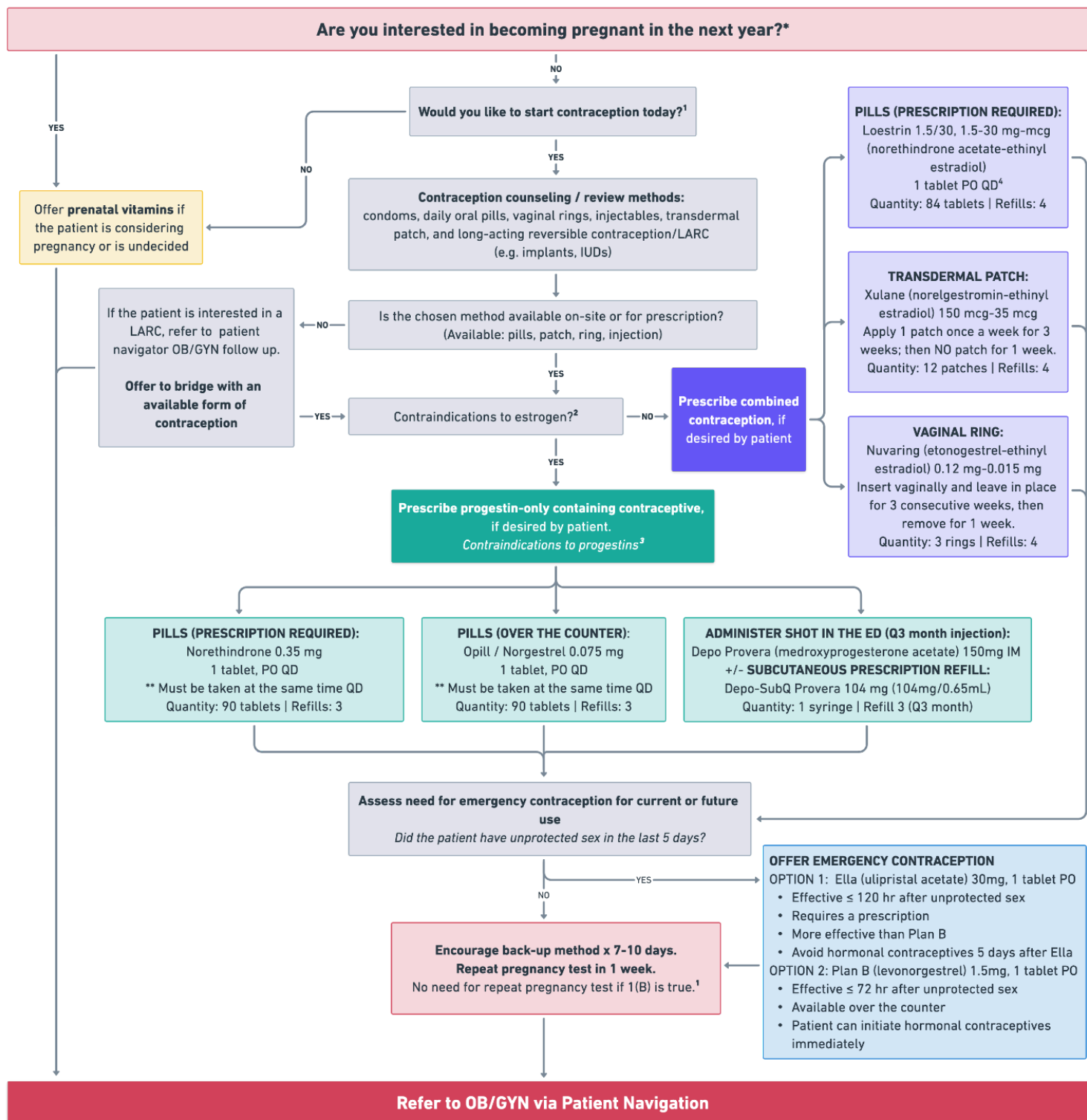


CONTRACEPTION INITIATION IN THE ED



* Inclusion criteria: consider initiating contraception in pregnancy-capable patients who are stable for discharge ED. Avoid initiation in patients with complicated medical histories.

1. **Rule out pregnancy.** Start contraception today if reasonably sure the patient is not pregnant:

(A) order a beta-hCG (serum if drawing blood) prior to initiating contraception. To avoid false negatives, follow (B)

(B) confirm the patient does not have pregnancy symptoms, AND one of the following: ≤7 days since the start of menses / miscarriage / abortion; no intercourse since last menses; uses contraception consistently; <4 weeks postpartum; breastfeeding ≥ 85% of the time; < 6 months postpartum and amenorrheic

2. **Contraindications to estrogen:** uncontrolled HTN, <6 weeks postpartum, breastfeeding <6 weeks postpartum, smoking AND ≥35 yo, thrombotic risk (hx of VTE, malignancy, CAD, stroke/TIA, PAD, anti-phospholipid syndrome, factor V leiden, etc.), SLE, RA, migraine w/ aura, uncontrolled DM, gastric bypass. Caution with some AEDs due to interactions.

3. **Contraindications to progestin contraception:** breast cancer, liver disease or tumors, gastric bypass surgery, on AEDs.

Note: Progestin-only pills are only slightly less effective than combined contraceptives.

4. If patient receives a 3 week pack, take one week off (no pills x 7 days) before starting a new pack. If the patient receives a 4-week pack, take 1 pill daily. You may also prescribe a different brand if patient prefers an alternative.

*** CDC Medical Eligibility Criteria for Contraceptive use → see QR code to review the safety of a contraceptive method based on medical conditions

